



Havana Specialist Hospital, Competency Check and the Live Mock Committee Meeting

The close-out of the governance training programme

Prepared for: The Board and Management of Havana Specialist Hospital Limited

Prepared by: Dr Debo Odulana, Independent Non-Executive Director; Consult for Africa

Faculty: Consult for Africa, with Barr. Gesiye Otuogha and Barr. Osahon Omoruyi

Companion to: the Governance Training Curriculum, the Board and Committee Instruments Pack, and the Board and Committee Handbook

Date: 16 August 2026

Classification: Confidential

The modules have been delivered. This is how the programme closes: a live mock committee meeting run on HSH's own instruments, a short competency check per cohort, and a one-page assurance to the People and Remuneration Committee. It is light and non-punitive by design. The purpose is not to grade anyone. It is so that the board can say, on evidence rather than assumption, that the capability is real before the September meeting.

1. Why a mock, and why this one

A curriculum can be delivered without changing a single meeting. The mock is the test that it landed, because it puts every taught element into one room at once: a chair holding time, a conflict declared and handled, a variance challenged, a paper that is not good enough, a decision reached and minuted, and a matter correctly pushed up to the board rather than settled locally.

We run it as the **Finance, Investment and Operations Committee**, deliberately. It is the committee that is co-chaired across both founding families, so it is the only one where fair co-chairing and the escalation of a genuine disagreement can be practised rather than described. Every element below has been planted to test a specific learning objective.

Format. Ninety minutes for the meeting, forty five for the debrief. Faculty observe and do not intervene, except that the Company Secretary may be asked a procedural question.

2. The mock meeting pack

Circulate this pack **seven clear days** before the mock, through the Company Secretary, exactly as a real pack would go out. Sending it late is itself part of the test, so do not send it late.

2.1 The scenario

It is the third quarter of the current financial year. The hospital has been trading under the Afya arrangement for two full quarters. Activity is recovering. The Finance, Investment and Operations Committee meets ahead of the board.



2.2 The agenda

HAVANA SPECIALIST HOSPITAL LIMITED
FINANCE, INVESTMENT AND OPERATIONS COMMITTEE
MOCK MEETING

Date: [date] Time: 90 minutes Co-chairs: [two directors]
Secretary: [Company Secretary] Quorum: 3

No.	Item	Lead	Type	Time
1.	Welcome, quorum, apologies	Co-chair	Noting	5
2.	Declarations of interest	Co-chair	Noting	5
3.	Minutes and action log	Sec	Decision	5
4.	Management accounts and Q3 performance	HoF	Discussion	25
5.	HMO receivables and the claims position	HoF	Discussion	15
6.	Paper FIOC/03: theatre imaging replacement	MD	Decision	25
7.	Operational KPIs by exception	Hosp Mgr	Noting	5
8.	What we send to the board	Co-chair	Decision	5

2.3 Item 4, the management accounts, extract

Line	Plan	Actual	Variance	Prior quarter
Revenue	N412m	N389m	(5.6%)	N371m
Payer mix, HMO share	45%	61%	+16 pts	48%
Payroll	N148m	N176m	+18.9%	N151m
Drugs and consumables	N71m	N78m	+9.9%	N69m
EBITDA	N58m	N19m	(67%)	N41m
Cash days on hand	45	26	(19 days)	38
Capital spend	N30m	N9m	(70%)	N12m

The narrative supplied with it reads, in full: "Revenue was slightly below plan owing to market conditions. Payroll reflects necessary investment in staffing to support activity growth. Management is monitoring the position closely and expects improvement next quarter."

2.4 Item 5, the HMO receivables position

Ageing	Amount	Share
0 to 30 days	N41m	22%
31 to 60 days	N38m	20%
61 to 90 days	N44m	24%
Over 90 days	N63m	34%
Total	N186m	



Claims submitted in the quarter: N204m. Claims rejected on first submission: N39m, 19 per cent. The rejection reason profile has not been supplied. Two HMOs account for 71 per cent of the balance over 90 days.

2.5 Item 6, board paper FIOC/03

Title: Replacement of the theatre imaging unit.

Sponsor: Medical Director.

Requested: approval to purchase a replacement intra-operative imaging unit at **N45m**, not in the approved capital budget.

Justification as written: *"The current unit is unreliable and has failed twice this quarter, causing three cancelled lists. The clinical team considers replacement urgent. The preferred supplier is Meridian Medical Systems, who can deliver in four weeks."*

The paper contains no options appraisal, no financial effect beyond the price, no risk section, and no measure of success. **Meridian Medical Systems is a company in which a member of one director's immediate family holds a directorship.** That fact appears in the register of interests but is not mentioned in the paper.

2.6 What is planted, and what each element tests

Element	Tests	Module
The related-party supplier in item 6	Spotting a conflict not flagged for you, declaring, recusing, and minuting it	M2, M8
The N45m unbudgeted capital request	Knowing this is a reserved matter that the committee recommends but cannot approve	M3
The empty management narrative at item 4	Refusing a report that explains nothing, and applying the four standing questions	M5
Payroll up 18.9 per cent while EBITDA falls 67 per cent	Connecting two numbers rather than reading them separately	M5, M7
Cash at 26 days against a 30-day threshold	Recognising a crossed escalation threshold that nobody escalated	M7
The missing claims rejection profile	Insisting on the information needed to decide, rather than deciding without it	M5
HMO concentration in the over-90 balance	Seeing a commercial risk inside a finance report	M7
The paper with no options and no success measure	Sending a paper back, courteously and firmly	M3, M5
Co-chairs who disagree on item 6	Escalating a genuine deadlock to the board instead of forcing a casting vote	M9
Item 8	Producing the one-page committee report to the board	M9

Faculty should not reveal this table until the debrief. Participants receive only the pack.



2.7 Roles

Role	Played by
Co-chairs	The two FIOC co-chairs, as in real life
Committee members	Directors, as in real life
Head of Finance	Management, presenting items 4 and 5
Medical Director	Presenting item 6
Hospital Manager	Presenting item 7
Company Secretary	Minuting live, as in real life
Observers	Faculty, silent, scoring against the rubric

The director whose family interest touches Meridian plays the part whether or not the interest is theirs in reality. If that is uncomfortable, faculty assigns it to a non-executive so nobody's real relationships are simulated.

2.8 What a good meeting produces

By the end, on the table, there should be: a minuted declaration and recusal; a management report sent back for rework with a specific list of what is missing; a decision **not** to approve item 6 at committee, with a recommendation to the board and the conflict handled; the cash position escalated to the Board Chair; and a signed one-page committee report. If those five exist, the training has landed.

3. The competency check

Light, non-punitive, and completed in the same session. Nobody fails; gaps generate a plan, not a sanction.

3.1 Cohort A, all directors

Part one, the scenario exercise. Thirty minutes, written individually, three short scenarios.

1. **A conflict to spot and handle.** You are sent a board paper recommending a supplier. Nothing in the paper flags it, but you recall the supplier's parent company shares a director with a company your fellow director chairs. What do you do, in what order, and what should appear in the minute?
2. **A variance to question.** Occupancy is up 12 per cent on plan and revenue is down 6 per cent. Write the four questions you ask, and say what answer would satisfy you.
3. **A decision to frame.** Management asks the board to approve opening a dialysis service. Write the one-sentence decision requested, and list the four things you must see before you would vote.

Part two, the self-assessment. Against the objectives of M1 to M8, each director rates themselves: *confident, workable, or want more support*. It is a request for support, not an examination, and it is seen only by the People and Remuneration Committee in aggregate.

Module	Capability	Confident	Workable	Want more
M1	CAMA, the company as a legal person, the three organs			



Module	Capability	Confident	Workable	Want more
M2	Directors' duties, liability, conflicts, minority protection			
M3	Governing versus managing, the HSH operating model			
M4	Compliant meetings, resolutions, minutes			
M5	Reading the board pack and challenging well			
M6	Clinical governance from the board's seat			
M7	Financial oversight, audit, internal control			
M8	Conflicts, fair process, the two-family dimension			

3.2 Cohort B, committee chairs

Assessed by chairing the mock, against this rubric. Scored 1 to 4 by two observers independently, then reconciled.

Criterion	1, developing	4, strong
Agenda and preparation	Agenda inherited, chair unprepared	Agenda shaped with the Secretary, chair has read everything and knows the two items that matter
Time discipline	Overruns, first item eats the meeting	Holds time, protects the decision items, parks tangents explicitly
Drawing out voices	The loudest speak, the quiet do not	Every member contributes, the chair invites silence into speech
Preventing domination	Executives or one family lead the room	Chair balances the floor without making it obvious
Handling the conflict	Missed, or handled awkwardly	Declared, recusal managed with dignity, minuted precisely
Challenge quality	Accepts the paper as presented	Sends it back with a specific, courteous list
Reaching a decision	Drifts, no clear outcome	Names the decision, tests agreement, closes it
Recording	Leaves it to the Secretary	Confirms the resolution wording aloud before moving on



Criterion	1, developing	4, strong
Escalation judgement	Settles what should go up	Recognises the reserved matter and routes it to the board
Committee report	Absent or narrative	One page, on the template, with a clear ask of the board

Standard expected: an average of 3 or above, with no criterion at 1. Co-chairs are scored jointly on the balance criteria and individually on the rest.

3.3 Cohort C, management and the secretariat

Two practical tasks, on the real instruments.

Task one, the exception report. Using Instrument 5 from the Instruments Pack and the mock data at sections 2.3 and 2.4, rewrite the management narrative. It must identify the three things the board should know, answer the four standing questions for every exception, and state the escalation that should already have happened on cash. Assessed against: does it explain rather than describe, is it three pages or fewer, does it name owners and dates, and would a director know what to do after reading it.

Task two, the meeting cycle. The Company Secretary runs one full cycle to standard: notice issued, agenda agreed with the chair, pack assembled and circulated seven clear days ahead, attendance and declarations recorded, minutes drafted within five working days, action log updated and carried forward, registers updated. Assessed on completeness and timeliness, not on prose.

4. The assurance to the People and Remuneration Committee

One page, prepared by the Company Secretary with faculty, submitted to the People and Remuneration Committee and noted by the board.



GOVERNANCE TRAINING PROGRAMME, COMPLETION ASSURANCE

To: People and Remuneration Committee
 From: Company Secretary, with the faculty
 Date:
 Programme: Board and management governance training, [dates]

1. COVERAGE

Modules delivered: [n] of 10
 Faculty: Barr. G. Otuogha, Barr. O. Omoruyi,
 Consult for Africa
 Cohort A attendance: [x] of 8 directors, [n] modules each
 Cohort B attendance: [x] of [y] committee chairs
 Cohort C attendance: [x] of [y] management and secretariat

2. COMPETENCY OUTCOME BY COHORT

Cohort A: [n] of 8 directors completed the scenario exercise and self-assessment. Objectives met: [n]. Gaps: [list].
 Cohort B: [n] chairs assessed on the mock. Average score [x] of 4. Any criterion scored 1: [list, with the chair unnamed if the committee prefers].
 Cohort C: exception report standard met by [n] of [y].
 Meeting cycle run to standard: yes / no.

3. THE MOCK MEETING

Held [date]. Of the five outcomes a good meeting produces, [n] were achieved. Not achieved: [list].

4. GAPS AND THE PLAN TO CLOSE THEM

Gap	Who	How	By when
[e.g. financial oversight confidence]	2 directors	Short refresher with the Audit chair	Q1 board

5. THE COMMITTEE'S ASSURANCE TO THE BOARD

The People and Remuneration Committee is / is not able to assure the board that each cohort has met its objectives, subject to the plan at section 4.

Signed: _____ Chair, People and Remuneration Committee

Directors are named in attendance and unnamed in scoring. The board needs to know the capability is there. It does not need a league table, and a league table would make the next round of honest self-assessment impossible.

5. What happens after the check

1. **The gaps become a plan**, owned by the People and Remuneration Committee, closed by the Q1 board.
2. **The instruments go live** at the September board meeting, which adopts the suite formally.
3. **The supported first cycle**, if the board takes the option, runs across the first quarter of real meetings, with faculty attending quietly and giving each chair private feedback.
4. **The real test is the September meeting**. Not that people sat through the programme, but that the meeting runs like the model: papers on time, conflicts declared, exceptions challenged, decisions minuted with reasons, and the Afya item taken as a standing matter.



Prepared by Dr Debo Odulana for the Board and Management of Havana Specialist Hospital. Consult for Africa. Confidential. The competency check is developmental and is not a performance assessment for any employment or office-holding purpose.